

إسم المريض رابعياً : خديجه صلاح الدين السيد ابراهيم

المحافظة : القاهرة

رقم البطاقة : 28208151604264 رقم التلفون : 01276008577 تاريخ تسجيل الإستمارة : 29-09-2026

العمر(تاريخ الميلاد) : 15-08-1982 المهنة : بدون عمل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج : الاورام

خطة العلاج : أبحاث أورام اشعه مقطعيه علي البطن والصدر والحوض بالصيغه [11127]

التعليق على التشخيص : -----

الاسم: د/محمد كيلاني	الوظيفة: -----	التوقيع:
الاسم: د/ندى حسن	الوظيفة: -----	التوقيع: -----
الاسم: د/نرمين رمضان	الوظيفة: -----	التوقيع: -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جھة العلاج :-

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشئون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: خديجه صلاح الدين السيد ابراهيم National ID: 28208151604264	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

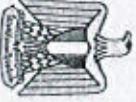
المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



 جمهورية مصر العربية
 وزارة الداخلية
 خديجة
 صلاح الدين السيد ابراهيم
 ٥١ ش النصر - ش الازهرام - ينجم
 شبرا الخيمة اول - القنوية
 ٢٨٢٠٨١٥١٦٠٤٢٦٤
 KJ3777803



 ٢٠٢٤/٠١ ٢٨٢٠٨١٥١٦٠٤٢٦٤
 حاصلة على دبلوم صناعي
 انش مملكة متروية
 وليد عبدالعظيم عبدالعظيم علي غانم
 البطاقة سارية حتى ٢٠٣٠/١٢/٣١


 ١١

A- Patient ID:

1- Name: **د. محمد عبد الله بن عبد الرحمن**

2- ID number: 28208151604264

Sex:

MDT form

3- Medical number:

83081352

Male

Patient Age: 40 Y

Family history

-VE

Phone

01206951482

BMI

HERS Score

GROUP

A

marital status

Menopausal status

PRE

Comorbidity

Date of HDT

12/20/2022

C) REASON OF HDT

POST LTHRM

Clinical data

Breast Imaging:

Mels Workup:

CICAP6/2022-->FREE

BX:

LT IDC G11

IHC:

ER:

8/8

PR:

8/8

Her 2 Neu:

1VE

KI67:

20%

Y

4D

N

2

M

0

Sonomammography

6/2022

LT MULTI FOCAL MALIGNANT MASS BIRADS 5

PATHOLOGICAL LT AXILLARY LNS

Recommendations:

LT BREAST CANCER POST NACT,HRM SPECIMEN:

TOTAL TUMOR REGRESSION IN A PREVIOUSLY DIAGNOSED INVASIVE CARCINOMA

FREE NIPPLE AND AREOLA WITH DERMAL SUPPURATIVE INFLAMMATORY REACTION

FREE DEEP MARGIN (LN 1/10)

Path. Par. Resp. YPT0 YPN1

MDT OUTCOME & PLAN

For: .PORT to Lt. chest wall, paraclav. & axilla.

.TDM1 for a total of 1 yr anti Her2 (14 cycles)

.Endocrinal TTT (Zoladex/AI for 5yrs).

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

Psychiatric

RADIOLOGY

PATHOLOGY

Nutrition

about blank



Patient Name : **الأستاذة د. منى محمد**
 Patient No. : **20208151604264**
 Refr No. : **3710/22**
 Sex : **Female**
 Age : **40 Y**
 Referring Doc :
 Request No. : **13005241**
 Request Date : **10/12/2022**
 Verification Date : **10/12/2022**
 Financial Category : **أورام دار السلام - مملوكة**
 Department : **Out Patient**
 Registered by : **marwa mohamed youss**

Pathology Report

Clinical Data :

Left breast cancer post NACT

Nature of the specimen : Right MRM specimen collected at 10/12/22

Gross Examination:

Left MRM specimen measured 19x17x7 cm , with covering skin ellipse measured 21x14 cm
 • Tumor extent : An ill defined rubbery greyish white tumor bed site
 • Size: 2x1.5x1cm
 • Skin involvement. Nipple & areola were unremarkable grossly with reddish skin lesion oozing yellowish fluid

• Distance from margins: 0.2 cm away from deep margin
 • Lymph nodes number and size: 10 LNs up to 1 cm.

Microscopic examination:

- Histological type: Total tumor regression in a previously diagnosed carcinoma
- Lymph node involvement. Positive (1/10)
- Skin involvement : Free nipple and areola
- Skin lesion showed dermal suppurative inflammatory reaction
- Resection Margins: Free deep margin
- Histologic response to therapy: Pathologic partial response

Diagnosis

- Left Breast cancer , post NACT , MRM specimen :
- Total tumor regression in a previously diagnosed invasive carcinoma
- Free nipple and areola , with dermal suppurative inflammatory reaction
- Free deep margin
- Lymph nodes. Positive (1/10)
- Pathologic partial response

Verified by :
 dr.esraa magdy

Printing Date : 11-12-2022 12:51:35

Printing By : marwa mohamed youss



Patient Name	الأستاذة / خديجة صلاح الدين السيد ابراهيم	Request No.	13008516
Patient No.	28208151601264	Request Date	06/06/2022
Path No.	1552/22	Verification Date	16/06/2022
Sex	Female	Financial Category	لام - ميسره
Age	39 Y	Department	Out Patient
Referring Doc.		Registered by	Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

- HER-2: Positive (score 3+)
- Ki67: Positive in 20% of tumor cells.

Dr. Walaa Ghonaim

Prof. Dr. Gamal Abdola
Dr. Israa Elgondy

Verified by :
walaa ghonaim

Printing Date : 18/06/2022 09:36:08

Page 2 of 2

Printed By : Ali mostafa



Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim

Date : 23/06/2022

Exam : C.T CHEST WITH CONTRAST

Age : 40 Year(s)

Medical ID:

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: :

A known case of breast cancer,

1. Chest:

- Left breast Heterogeneously enhanced parenchyma with overlying thickened skin. Bilateral axillary enlarged lymph nodes the largest at left side measures about 13 X 9 mm.
- No sizable pulmonary nodules.
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged showing homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No Intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No lytic or sclerotic bone lesions.

OPINION:

- *Left breast Heterogeneously enhanced parenchyma with overlying thickened skin and bilateral axillary enlarged lymph nodes.*
- *No CT evidence of pulmonary or hepatic metastatic deposits.*

Dr. Ayat Nadi

Dr. M. G. Gamaa, M.D.
CT Scan
[[DOCTYPE:CT_SCAN]]



Patient Name: Khadija Solaheldeen Elsayed.

Date: 3/3/2022

^{99m}Tc -MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP.

Report:

- Warm uptake is seen involving the right femur and right tibia likely post internal fixation and intervention for clinical and radiological correlation as well as follow up.
- Otherwise, no definite hot spot that account for osseous deposits could be detected all over the rest of surveyed skeleton.
- Degenerative changes are noted in the dorso-lumbar vertebrae.
- Both kidneys are seen

Conclusion:

- *No definite evidence of metastatic osseous deposits.*
- *Clinical and radiological correlation of the right femur and tibia is recommended.*

Thanks for reference
Nuclear Medicine Unit

DR/Huda Fathy, MD

[illegible]



Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 29/08/2024
Exam : MAMMOGRAPHY BOTH	Age : 42 Year(s)
Medical ID: 2288/2022	Pat. ID: 12669

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is scattered fibroglandular densities of the right breast (ACR B)
- Right UIQ area of asymmetrical density with no underlying spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- Right breast UIQ area of asymmetry appreciated more in CC view for follow up after 6 months by single view mammogram (BIRADS 3)

Yours truly

Signature Dr. Asmaa Zenhom

29/08/2024 3:53 PM



Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 23/06/2022
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID:	

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A known case of breast cancer.

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- No sizable pulmonary nodules.
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- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No lytic or sclerotic bone lesions.

OPINION:

- ***Left breast Heterogeneously enhanced parenchyma with overlying thickened skin and bilateral axillary enlarged lymph nodes.***
- ***No CT evidence of pulmonary or hepatic metastatic deposits.***

Dr. Ayat Nadi

Dr. M. Gomaa, M.D.

Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 05/08/2025
Exam : MAMMOGRAPHY BOTH	Age : 43 Year(s)
Medical ID: 2288/2022	Pat. ID: 12669

**FOLLOW UP RIGHT BREAST MAMMOGRAPHY WITH
BILATERAL ULTRASOUND**

INDICATION:

- Left MRM

TECHNIQUE:

- Right breast MLO and CC views
- Bilateral ultrasound

REPORT:

- Fairly dense parenchyma, (ACR B), with the focal tissue island still seen in the upper midline aspect of the breast.
- No spiculated masses or grouped microcalcifications.
- Complementary U/S also showed no solid or cystic masses, only underlying condensation of parenchyma.
- Normal skin thickness and nipple-areola complex.
- Right nonspecific axillary nodes, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions, and free infra and supra clavicular regions.

OPINION:

- Left MRM with free operative bed.
- Stationary right benign finding, (BIRADS 2), for follow up.

**Best regards,
Dr. Mahi Mokhtar**



Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 23/11/2022
Exam : MAMMOGRAPHY BOTH	Age : 40 Year(s)
Medical ID: 2288/2022	Pat. ID: 12669

Full Field Digital bilateral breast mammography & Complementary Ultrasound

Indication:

left breast pathologically proved carcinoma on chemotherapy.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Diffuse Left breast edema pattern with UOQ spiculated irregular mass lesion with adjacent pleomorphic calcifications, of high density.
- Complementary ultrasound revealed left breast no spiculated mass lesion seen in UOQ, stationary course of UOQ suspicious microcalcifications.
- Normal skin thickness, and nipple-areola complex of the right breast.
- One left still noted with cortical thickening reaching to 5mm in level I.
- supraclavicular pathological lymph nodes.
- Nonspecific right axillary LN.

Opinion:

- Left breast pathologically proved carcinoma on chemotherapy with good response in ultrasound but still noted UOQ malignant microcalcifications and still noted one pathological axillary lymph node at level I, (BIRADS 6)
- Normal right breast (BIRADS 1)
-

Signature Dr. Nehal Hamdy

23/11/2022 12:35 PM



Presidential Program for Women's health

Name: Khadija salaheddin elsayed

Date: 7/7/2022

Targeted Bilateral axillary Ultrasound

Clinical summary: left breast pathologically proved multicentric IDC

Technique

High resolution ultrasound with Color Doppler.

Detailed Findings

Left breast

- Dense fibro glandular breast parenchyma.
- Second look ultrasound showed multicentric malignant looking variable size mass lesions scattered all over the area between 10 o'clock till 5 o'clock, involving the axillary tail of the breast. Scattered dilated ducts of involved areas are noted suggesting intraductal extension.
- Thickened skin/nipple/areolar complex. Reaching 7 mm
- Multiple Enlarged Pathological lymph nodes with thickened cortex reaching 10 mm with eccentric hilum , involving Level I and II including the infraclavicular lymph nodes, with 2 suspicious visualized supraclavicular lymph nodes, showing cortical thickness of 2 mm.

Right breast

- Dense fibroglandular breast parenchyma.
- No solid or cystic lesions.
- Normal skin/nipple/areolar complex.
- Axillary non-specific nodes.

Conclusion:

Left breast (BIRADS 6) multicentric IDC, with pathological axillary lymph nodes involving Level I and II with suspicious supraclavicular lymph nodes .
Right breast (BIRADS 1) non-specific axillar lymph nodes with Max cortical thickness of 2.8 mm .

DR, Eman Elmaraghi, FEBR

Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 05/06/2022
Exam : ultrasound BREAST (BOTH SIDE)	Age : 39 Year(s)
Medical ID: 969	

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Diffuse Left breast edema pattern with UOQ spiculated irregular mass lesion with adjacent pleomorphic calcifications, of high density.
- Complementary ultrasound revealed left breast multiple irregular spiculated hypoechoic mass lesions seen at 2 , 4 , 11 and 12 o'clock largest seen at 2 o'clock measure 3.7 x 2 cm and 12 o'clock measures 3 x 1.2 cm
- Normal skin thickness, and nipple-areola complex of the right breast.
- Pathological left axillary level I, II and III lymph nodes largest at level I measures 2.8 x 1.4 cm
- No supraclavicular pathological lymph nodes.
- Nonspecific right axillary LN.

Opinion:

- Left breast multifocal malignant looking mass lesions (BIRADS 5) for histopathological correlation.
- Pathological left axillary LNs.
- Normal right breast (BIRADS 1)

Signature

Dr.

Yours truly

Signature

Dr.

of the

radiologist

Signature Dr. Aya Deabes

05/06/2022 03:13



NUCLEAR MEDICINE UNIT

وحدة الطب النووي

Patient Name: Khadiga Salaheldeen Elsayed.

Date: 3/8/2022

^{99m}Tc-MDP BONE SCAN

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Nuclear Medicine Unit

DR/Huda Fathy, MD

Mark

ju

juv

John

John

ju

you

gr

Jan

you

g

g.

EF	LVEDD	LVEDS	FS	SWT	Dimensions
62	4.6	2.8	37	0.8	55-75%
	3.5-5.8cm	2-4cm	25-45%	0.6-1.1cm	PWT
					AO.R
					LA
			3.2	—	RV
				—	AO.CUSP
					1.5-2.6cm
					0.7-2.3cm(supine)
					2-4cm
					2-3.7cm
					0.6-1.1cm

mode

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١٥٨٩
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Handwritten signature and date 10/10/2020.

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة والسكان
مركز التخصص في أمراض السرطان
دار السلام
القاهرة

Patient Name: Khadiga Salah Al Din Al Sayed		Date : 29/08/2024
Ibrahim		Age : 42 Year(s)
Exam : MAMMOGRAPHY BOTH		Pat. ID: 12669
Medical ID: 2288/2022		

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Yours truly

Signature Dr. Asmaa Zenhom

29/08/2024 3:53 PM



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٢٨٢٠٨١٥١٦٠٤٢٦٤
٩٠٢١/٥٠١
١١

حاصلة على دبلوم صناعات
النسيج
معلمة
وليد عبدالمطلب عبدالمطلب علي غانم
البطاقة سارية حتى ٢٠٢٠/١٢/٣١

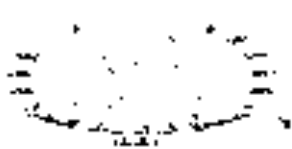


جمهورية مصر العربية
بطاقة تحقيق الشخصية

خديجة
صلاح الدين السيد ابراهيم
٥١ ش النصر - ش الاهرام - بيجم
شبرا القبة اول - القليوبية

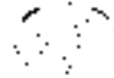


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KJ3777803



Preventive Program for Women's Health
Ministry of Health and Population

البرنامج الوقائي لصحة المرأة
وزارة الصحة والسكان



MDT form

A- Patient ID:

1- Name: علي محمد علي محمد

2- ID number: 28208151604264

Sex:

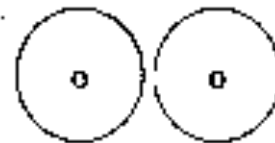
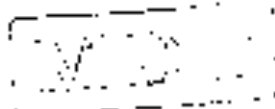
3- Medical number:

83081352

Male

Patient Age: 40 Y

CCNA
C.C.C



Family History

-VE

Phone

01208881482

BMT

MRS Score

GROUP

A

marital status

Menopausal status

PRE

Co morbidity

Date of MDT

12/29/2022

REASON OF MDT

POST LT MRM

Clinical data

Breast Imaging:

Mats Workup:

CTCAP6/2022 -> FREE

BX:

LT IDC G1

IHC:

ER:

8/B

PR:

8/B

Her 2 Neu:

+VE

K167:

20%

T

4D

N

2

M

0

Sonomammography

6/2022

LT MULTI FOCAL MALIGNANT MASS BIRADS 5

PATHOLOGICAL LT AXILLARY LNS

Recommendations:

LT BREAST CANCER POST NACT, MRM SPECIMEN:

TOTAL TUMOR REGRESSION IN A PREVIOUSLY DIAGNOSED INVASIVE CARCINOMA

FREE NIPPLE AND AREOLA WITH DERMAL SUPPURATIVE INFLAMMATORY REACTION

FREE DEEP MARGIN (LN 1/10)

Path. Par. Resp. YPT0 YPN1

MDT OUTCOME & PLAN

For: PORT to Lt. chest wall, paraclav. & axilla.

TDM1 for a total of 1 yr anti Her2 (14 cycles)

Endocrinal TTT (Zoladex/AI for 5yrs).

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

Psychiatric

RADIOLOGY

PATHOLOGY

Nutrition



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الصحة المصرية
Ministry of Health
MDT FORM

MDT FORM

23/ 11/2022 GA

Name	Dr. Mohamed Elmaghrabi	MRN	2020-11-17-1072
Age	39 years old	ID	55515544
Menopausal status	perimenopausal	Phone	01557777777
NRS SCORE		Marital status	married
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX:	IDA G II	R	L
ER:	PR:	HER 2:	Ki67:
8/8	2/8	+	20%
Sonomammography:			
Mets Workup:			
received 6 T C & 4 HP			
TNM			
was clinically staged			
now much subjective improvement			

Recommendations:

for amputation regarding it MRM

as per initial MDT

Ref STN → requested 11/12

MDT OUTCOME & PLAN

For left MRM per initial decision.

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 29/08/2024
Exam : MAMMOGRAPHY BOTH	Age : 42 Year(s)
Medical ID: 2288/2022	Pat. ID: 12669

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique:

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is scattered fibroglandular densities of the right breast (ACR B)
- Right UIQ area of asymmetrical density with no underlying spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- Right breast UIQ area of asymmetry appreciated more in CC view for follow up after 6 months by single view mammogram (BIRADS 3)

Yours truly

Signature Dr Asmaa Zenhom

29/08/2024 3:53 PM



Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 23/06/2022
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID:	

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

A known case of breast cancer.

1. Chest:

- Left breast Heterogeneously enhanced parenchyma with overlying thickened skin. Bilateral axillary enlarged lymph nodes the largest at left side measures about 13 X 9 mm.
- No sizable pulmonary nodules.
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged showing homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No lytic or sclerotic bone lesions.

OPINION:

- *Left breast Heterogeneously enhanced parenchyma with overlying thickened skin and bilateral axillary enlarged lymph nodes.*
- *No CT evidence of pulmonary or hepatic metastatic deposits.*

Dr. Ayat Nadi

Dr. M. Gomaa, M.D.

Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 05/08/2025
Exam : MAMMOGRAPHY BOTH	Age : 43 Year(s)
Medical ID: 2288/2022	Pat. ID: 12669

**FOLLOW UP RIGHT BREAST MAMMOGRAPHY WITH
BILATERAL ULTRASOUND**

INDICATION:

- Left MRM

TECHNIQUE:

- Right breast MLO and CC views
- Bilateral ultrasound

REPORT:

- Fairly dense parenchyma, (ACR B), with the focal tissue island still seen in the upper midline aspect of the breast.
- No spiculated masses or grouped microcalcifications.
- Complementary U/S also showed no solid or cystic masses, only underlying condensation of parenchyma.
- Normal skin thickness and nipple-areola complex.
- Right nonspecific axillary nodes, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions, and free infra and supra clavicular regions.

OPINION:

- Left MRM with free operative bed.
- Stationary right benign finding, (BIRADS 2), for follow up.

**Best regards,
Dr. Mahi Mokhtar**

Patient Name: Khadiga Salah A. Din Al Sayed Ibrahim	Date : 23/11/2022
Exam : MAMMOGRAPHY BOTH	Age : 40 Year(s)
Medical ID: 2288/2022	Pat. ID: 12669

Full Field Digital bilateral breast mammography & Complementary Ultrasound

Indication:

left breast pathologically proved carcinoma on chemotherapy.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Diffuse Left breast ecema pattern with UOQ spiculated irregular mass lesion with adjacent pleomorphic calcifications, of high density.
- Complementary ultrasound revealed left breast no spiculated mass lesion seen in UOQ, stationary course of UOQ suspicious microcalcifications
- Normal skin thickness, and nipple-areola complex of the right breast.
- One left still noted with cortical thickening reaching to 5mm in level
- supraclavicular pathological lymph nodes.
- Nonspecific right axillary LN

Opinion:

- Left breast pathologically proved carcinoma on chemotherapy with good response in the UOQ but still noted UOQ malignant microcalcifications and still noted one pathological axillary lymph node at level I, (BIRADS 5).
- Normal right breast (BIRADS 1)
-

Signature Dr. Nehal Hamdy

23/11/2022 12:35 PM



Presidential Program for Women's health

Name: Khadija salaheddin elsayed

Date: 7/7/2022

Targeted Bilateral axillary Ultrasound

Clinical summary. left breast pathologically proved multicentric IDC

Technique

High resolution ultrasound with Color Doppler.

Detailed Findings

Left breast

- Dense fibro glandular breast parenchyma.
- Second look ultrasound showed multicentric malignant looking variable size mass lesions scattered all over the area between 10 o'clock till 5 o'clock, involving the axillary tail of the breast. Scattered dilated ducts of involved areas are noted suggesting intraductal extension.
- Thickened skin/nipple/areolar complex. Reaching 7 mm
- Multiple Enlarged Pathological lymph nodes with thickened cortex reaching 10 mm with eccentric hilum , involving Level I and II including the infraclavicular lymph nodes, with 2 suspicious visualized supraclavicular lymph nodes, showing cortical thickness of 2 mm

Right breast

- Dense fibroglandular breast parenchyma.
- No solid or cystic lesions.
- Normal skin/nipple/areolar complex.
- Axillary non-specific nodes.

Conclusion:

Left breast (BIRADS 6) multicentric IDC, with pathological axillary lymph nodes involving Level I and II with suspicious supraclavicular lymph nodes .

Right breast (BIRADS 1) non-specific axillar lymph nodes with Max cortical thickness of 2.8 mm .

DR, Eman Elmaraghi, FEBR

Patient Name: Khadiga Salah Al Din Al Sayed Ibrahim	Date : 05/06/2022
Exam : ultrasound BREAST (BOTH SIDE)	Age : 39 Year(s)
Medical ID: 969	

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Diffuse Left breast edema pattern with UOQ spiculated irregular mass lesion with adjacent pleomorphic calcifications, of high density.
- Complementary ultrasound revealed left breast multiple irregular spiculated hypoechoic mass lesions seen at 2 , 4 , 11 and 12 o'clock largest seen at 2 o'clock measure 3.7 x 2 cm and 12 o'clock measures 3 x 1.2 cm
- Normal skin thickness, and nipple-areola complex of the right breast.
- Pathological left axillary level I, II and III lymph nodes largest at level I measures 2.8 x 1.4 cm
- No supraclavicular pathological lymph nodes.
- Nonspecific right axillary LN.

Opinion:

- Left breast multifocal malignant looking mass lesions (BIRADS 5) for histopathological correlation.
- Pathological left axillary LNs.
- Normal right breast (BIRADS 1)

Dr. Aya Deabes

Dr. Aya Deabes

Yours truly

Dr. Aya Deabes

Signature Dr. Aya Deabes

Dr. Aya Deabes

Dr. Aya Deabes

Dr. Aya Deabes

05/06/2022 03:13



NUCLEAR MEDICINE UNIT

وحدة الطب النووي

Patient Name: Khudiga Salahaideen Elsayed.

Date: 3/8/2022

^{99m}Tc -MDP BONE SCAN

Desired information : skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after 1.7 administration of 20 mCi ^{99m}Tc -MDP.

Report:

- Warm uptake is seen involving the right femur and right tibia likely post internal fixation and intervention for clinical and radiological correlation as well as follow up.
- Otherwise, no definite hot spot that account for osseous deposits could be detected all over the rest of surveyed skeleton.
- Degenerative changes are noted in the dorso-lumbar vertebrae.
- Both kidneys are seen

Conclusion:

- *No definite evidence of metastatic osseous deposits.*
- *Clinical and radiological correlation of the right femur and tibia is recommended.*

Thanks for reference
Nuclear Medicine Unit

DR/Huda Fathy, MD



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وزارة الصحة والرفاهية
مبنى مركز القلب والسرطان
مبنى أمراض القلب

وزارة الصحة والرفاهية
مبنى المراكز الطبية المتخصصة
مركز أمراض الدم

خبرته في أمراض القلب

اسم المريض :
رقم الملف :
نوع التوجيه :
تاريخ التوجيه :
وقت التوجيه :

Report

EF	67	55-75%	PWT	0.8	0.6-1.1cm
LVESD	4.6	3.5-5.8cm	AOR	2.6	2-3.7cm
LVESD	2.8	2-4cm	LA	3.2	2-4cm
FS	37	25-45%	RV	—	0.7-2.3cm (supine)
SWT	0.8	0.6-1.1cm	AO CUSP	—	1.5-2.6cm

LV dimensions *normal*
 LV wall thickness *normal*
 LV diastolic function *normal*
 SWMA *normal*
 LA and Ao root dimensions *normal*
 Mitral Valve *normal*
 Aortic valve *normal*
 Tricuspid valve *normal* *Tricuspid TR, normal RV*
 Pulmonary valve *normal*
 Right side and systolic function *normal*
 Intracardiac masses and effusion *normal*

Mohammed Anwar, MD cardiologist

[Signature]